

Lid swelling alone does not locate the infection

Painful restricted movements, proptosis or optic neuropathy support postseptal disease.

Locate the infection

Look beyond the swollen eyelids

Preseptal

Lid inflammation; normal painless eye movements, no proptosis or optic nerve signs.

Orbital

Painful restricted EOM, proptosis and possible RAPD or reduced vision.

Clinical meaning

Orbital cellulitis often follows ethmoidal sinusitis. Admit urgently, start IV broad-spectrum antibiotics and involve ophthalmology and ENT. CT assesses infection extent and abscess. Monitor acuity, colour, pupils and pressure; worsening vision or treatment failure needs escalation. Intracranial spread is a serious risk, so outpatient oral therapy is insufficient for threatened vision.

TED has both surface and deep orbital consequences

New visual or colour loss in TED should prompt assessment for optic nerve compression.

TED structures

Structure	Manifestation
Lids and cornea	Retraction or incomplete closure causes exposure.
Extraocular muscles	Enlargement and fibrosis cause restrictive diplopia.
Orbit	Expanded contents cause proptosis.
Optic nerve	Apical compression reduces acuity, colour or field.

Clinical meaning

Tendon-sparing muscle enlargement supports TED on imaging. Assess thyroid status and stop smoking. Protect an exposed cornea with lubrication and lid measures. New optic neuropathy is an emergency: urgent systemic steroids and decompression assessment if response is inadequate. Thyroid control or a prism cannot replace assessment of deteriorating vision.